

1. After surgery, Ms. Hanna is having difficulty voiding. Which nursing action would most likely lead to an increased difficulty with voiding?

- a. Using a warm bedpan when Ms. Hanna feels the urge to void
- b. Having Ms. Hanna ignore the urge to void until her bladder is full
- c. Stroking Ms. Hanna's leg or thigh
- d. Pouring warm water over Ms. Hanna's fingers

2. Nursing care for a patient with an indwelling catheter includes which of the following?

- a. Telling the patient that burning and irritation are normal, subsiding within a few days
- b. Encouraging a generous fluid intake if not contraindicated by the patient's condition.
- c. Irrigation of the catheter with 30 mL of normal saline solution every 4 hours
- d. Disconnecting and reconnecting the drainage system quickly to obtain a urine specimen

3. A 28-year-old woman who is 6 months pregnant presents to the emergency department with a temperature of 38.2°C (100.8°F) and complains of shaking chills and pain on her right side, which she locates by pointing to the area above her right iliac crest. During the examination she is tender to percussion at the junction of the lower ribs and the thoracic vertebrae. Urinalysis reveals WBC casts. Which of the following is the most likely diagnosis?

- a. Pyelonephritis
- b. Localized cystitis
- c. Ectopic pregnancy
- d. Appendicitis

4. In planning nursing interventions to increase bladder control in the patient with urinary incontinence, the nurse includes:

- a. Teaching the patient biofeedback mechanisms to suppress the urge to void
- b. Counseling the patient concerning choice of incontinence containment device.
- c. Teaching the patient to use Kegel exercises
- d. Clamping and releasing a catheter to increase bladder tone

5. The physician has ordered an indwelling catheter inserted in a hospitalized male patient. The nurse is aware of which of the following considerations?

- a. In the hospital a clean technique is used for catheter insertion
- b. The male urethra is more vulnerable to injury during insertion
- c. The catheter is inserted 2 to 3 cm into the meatus
- d. Since it used a closed system, the risk for urinary tract infection is absent

6. A patient with BPH suddenly experiences an inability to urinate, despite feeling a strong urge. This condition is known as:

- a) Stress incontinence
- b) Overflow incontinence
- c) Acute urinary retention
- d) Urethral stricture

7. Which electrolyte imbalance is a common and dangerous complication of advanced renal failure?

- a) Hypokalemia
- b) Hyponatremia
- c) Hypocalcemia
- d) Hyperkalemia

8. Which of the following is a common cause of chronic glomerulonephritis leading to end-stage renal disease?

- a) Acute pyelonephritis
- b) Polycystic kidney disease
- c) IgA nephropathy
- d) Uncomplicated cystitis

9. What is the primary route by which bacteria typically enter the urinary tract to cause a UTI?

- a) Hematogenous spread
- b) Lymphatic spread
- c) Ascending route
- d) Direct extension from adjacent organs

10. Which of the following symptoms is most indicative of pyelonephritis rather than simple cystitis?

- a) Suprapubic tenderness
- b) Painful urination
- c) Costovertebral angle (CVA) tenderness
- d) Urinary urgency

11. Which of the following is a classic clinical manifestation of acute glomerulonephritis (AGN)?

- a) Polyuria and polydipsia
- b) Gross hematuria and periorbital edema
- c) Severe flank pain radiating to the groin
- d) Profound hyperglycemia

12. What is the most common "intrinsic" (intra-renal) cause of AKI?

- a) Benign prostatic hyperplasia (BPH)
- b) Acute tubular necrosis (ATN)
- c) Nephrolithiasis
- d) Renal artery stenosis

13. Which medication class for BPH works by shrinking the prostate gland over time?

- a) Alpha-1 adrenergic blockers
- b) Anticholinergics
- c) 5-alpha reductase inhibitors
- d) Beta-blockers

14. The nurse is providing dietary teaching to a patient with chronic kidney disease (CKD) who is not yet on dialysis. Which dietary modification is most appropriate?

- A. High protein, low carbohydrate
- B. Low protein, high potassium
- C. Low protein, low potassium
- D. High protein, high sodium

15. A patient with nephrolithiasis is prescribed allopurinol. Which lab value will the nurse monitor to determine the medication's effectiveness?

- A. Blood urea nitrogen (BUN)
- B. Serum uric acid
- C. Serum calcium
- D. Serum creatinine

16. A nurse is assessing a patient with chronic kidney disease who has developed metabolic acidosis. Which of the following findings is most consistent with this condition?

- A. Bradypnea
- B. Decreased deep tendon reflexes
- C. Kussmaul respirations
- D. Hypertension

17. You're preparing for urinary catheterization of a trauma patient and you observe bleeding at the urethral meatus. Which action has priority?

- A. Irrigate and clean the meatus before catheterization.
- B. Check the discharge for occult blood before catheterization.
- C. Heavily lubricate the catheter before insertion.
- D. Delay catheterization and notify the doctor.

18. A patient returns from surgery with an indwelling urinary catheter in place and empty. Six hours later, the volume is 120ml. The drainage system has no obstructions.

Which intervention has priority?

- A. Give a 500 ml bolus of isotonic saline.
- B. Evaluate the patient's circulation and vital signs.
- C. Flush the urinary catheter with sterile water or saline.
- D. Place the patient in the shock position and notify the surgeon.

19. A 28 years student is admitted with dark urine, fever, and flank pain and is diagnosed with acute glomerulonephritis. Which would most likely be in this student's health history?

- A. Renal calculi.
- B. Renal trauma.
- C. Recent sore throat.
- D. Family history of acute glomerulonephritis

20. The nurse is taking the history of a client who has had benign prostatic hyperplasia in the past. To determine whether the client currently is experiencing difficulty, the nurse asks the client about the presence of which of the following early symptoms?

- A. Urge incontinence.
- B. Nocturia.
- C. Decreased force in the stream of urine.
- D. Urinary retention

21. Mr X experiences severe flank pain came in to the hospital for further evaluation, and was diagnosed with renal calculi. His temperature is 38.4°C, rated the pain as severe, and is complaining of nausea. Which of the following would be a priority outcome for the patient?

- A. Prevention of urinary tract complications.

- B. Alleviation of nausea
- C. Alleviation of pain.
- D. Maintenance of fluid and electrolyte balance

22. A Client with acute renal failure is experiencing a continuous elevation of serum potassium. The nurse anticipates this emergency situation to Occur:

- A. Cardiac arrest
- B. Pulmonary edema
- C. Circulatory collapse
- D. Cerebrovascular accident

23. A client with kidney failure experiences tingling of the fingers and toes. Muscular twitching is also noted. The nurse knows that this is caused by:

- A. Acidosis.
- B. Calcium depletion
- C. Potassium retention
- D. Sodium chloride depletion

24. A patient with nephritis reports fatigue and shortness of breath. On assessment, the nurse notes periorbital edema and crackles in the lungs. Which complication is the nurse most concerned about?

- A. Pulmonary embolism
- B. Acute kidney injury
- C. Fluid overload
- D. Urinary tract infection

25. A female adult client admitted with a gunshot wound to the abdomen is transferred to the intensive care unit after an exploratory laparotomy. Which assessment finding suggests that the client is experiencing acute renal failure (ARF)?

- A. Blood urea nitrogen (BUN) level of 22 mg/dl.
- B. Serum creatinine level of 1.2 mg/dl
- C. Serum potassium level of 4.1mEq/L
- D. Urine output of 400 ml/24 hours

26. A patient was taking high dose aspirin for long period of time and has developed acute renal failure. This form of renal failure is known as:

- A. Pre renal failure.
- B. Intra renal failure.
- C. Post renal failure.
- D. Pre- intra renal failure

27. A client arrives at the emergency department with complaints of lower abdominal pain and hematuria. The client has no fever. The nurse next assesses the client to determine a history of which condition?

- A. Pyelonephritis
- B. Glomerulonephritis
- C. Trauma to the bladder or abdomen
- D. Renal cancer in the client's family

28. The nurse is reviewing a client's record and notes that the health care provider has documented that the client has chronic renal disease. On review of the laboratory results, the nurse most likely would expect to note which finding?

- A. Elevated creatinine level
- B. Decreased hemoglobin level
- C. Decreased red blood cell count
- D. Increased number of white blood cells in the urine

29. The nurse is caring for a client with pre renal Acute Kidney injury(AKI). Which of the following conditions would most likely contribute to pre renal AKI?

- A. Bilateral kidney stones
- B. Hypotension
- C. Bladder obstruction
- D. Nephrotoxic medications

30. A nurse is preparing to administer medications to a client with AKI.

Which medication order should the nurse question?

- A. IV furosemide (Lasix) to treat fluid overload
- B. IV vancomycin for an infection
- C. Oral sodium bicarbonate for metabolic acidosis
- D. Oral phosphate binder to manage hyperphosphatemia

31. A nurse is caring for a client with a urinary catheter. The nurse notices that the catheter's drainage bag is not filling with urine, and the client has not voided for several hours. Which of the following actions should the nurse take first?

- A. Irrigate the catheter with sterile saline.
- B. Reposition the client to ensure proper catheter alignment.
- C. Measure the client's abdominal girth to assess for bladder distension.
- D. Notify the healthcare provider about the potential obstruction.

32. A nurse is caring a patient admitted to medical ward with a diagnosis of acute kidney injury has history of confusion and irritable, The main cause of this behavior should be?

- A. Increased serum potassium level
- B. Decrease fluid and electrolyte balance
- C. Increased serum calcium level
- D. Increased blood urea-nitrogen level